# The Rise, Evolution, and Dissolution of the Public Utility Model: An Exhaustive History of MedStar Mobile Healthcare in Fort Worth (1986–2025)

## Abstract

This report provides a comprehensive historical and economic analysis of the Metropolitan Area EMS Authority (MAEMSA), operating as MedStar Mobile Healthcare, from its inception in the regulatory vacuum of the 1980s to its dissolution and absorption into the Fort Worth Fire Department (FWFD) in 2025. This document serves as a case study in the lifecycle of the "Public Utility Model" (PUM) of Emergency Medical Services (EMS). It examines the transition from unregulated competition to a high-performance regulated monopoly, the pioneering of Mobile Integrated Healthcare (MIH), and the eventual collapse of the fee-for-service funding structure under the weight of post-pandemic macroeconomic pressures. Through a detailed review of municipal ordinances, financial audits, and strategic reports, this analysis elucidates the tension between clinical innovation, economic sustainability, and political governance in the provision of prehospital care.

## Part I: The Genesis of Order – The Pre-MedStar Era and the Theoretical Framework (1978–1986)

### 1.1 The Market Failure of the "Ambulance Wars"

To comprehend the structural necessity of MedStar's creation, one must first dissect the chaotic ecosystem of prehospital care that characterized Fort Worth, Texas, in the late 1970s and early 1980s. During this period, the prevailing economic philosophy regarding ambulance service was one of deregulated free-market competition. Municipal leaders operated under the assumption that open competition among private ambulance providers—such as Gold Cross, various funeral home-based services, and smaller independent operators—would naturally drive down costs for patients while incentivizing rapid response times.

However, the application of standard market dynamics to emergency medicine proved disastrously flawed. The ambulance market of the early 1980s suffered from a profound disconnect between service demand and revenue potential. In a free market, private entities are naturally incentivized to pursue profit. In the context of EMS, this manifested as "cream skimming"—a practice where competing agencies aggressively targeted lucrative calls, such as scheduled inter-facility transfers (IFTs) or emergencies involving insured patients in affluent neighborhoods, while actively avoiding or delaying responses to indigent populations or geographically isolated areas.

The "Ambulance Wars" were not merely a metaphor; they were operational realities. Without a central authority or unified dispatch system, private companies monitored police scanners,

racing each other to accident scenes to "jump" calls and claim the billing rights for the patient. This led to dangerous driving behaviors, physical altercations between rival crews at scenes, and, most critically, a fragmentation of care. The system lacked accountability; no single entity was responsible for the city-wide standard of care. If a patient in a low-income bracket called for help, response times were often erratic, as private providers had no contractual obligation or financial incentive to prioritize non-paying customers.

By 1985, the situation in Fort Worth had reached a crisis point. The incumbent providers were deemed "extremely unreliable," with the media documenting instances of prolonged wait times and operational failures. The City Council faced a stark realization: the free market could not guarantee the universal safety net required for public health. Emergency medical care was a natural monopoly, not a commodity to be traded on the open market.

## 1.2 The Jack Stout Philosophy: Architecting the Public Utility Model

Enter Jack Stout, an economist from the University of Oklahoma who would fundamentally reshape the global landscape of EMS system design. Stout, observing the failures of both the chaotic private market and the inefficient, tax-heavy municipal models of the era, proposed a "Third Way": The Public Utility Model (PUM).

Stout's theory was rooted in the economic concept of the "natural monopoly." He argued that because the demand for emergency service is inelastic (a cardiac arrest patient cannot shop for a better price) and the readiness costs are fixed (an ambulance must be staffed and fueled regardless of whether it runs a call), competition *in* the market was destructive. It diluted call volume across too many providers, preventing any single agency from achieving the economies of scale necessary to afford advanced clinical equipment and high wages.

Instead, Stout advocated for competition *for* the market. Under the PUM, a local government would create a public authority to hold the exclusive market rights. This authority would then contract with a private company to operate the system. The contractor would be selected through a rigorous bidding process, winning the monopoly for a fixed term (usually 5 years). This structure combined the efficiency and expertise of the private sector with the accountability and equity of the public sector.

**The Pillars of the Stout Model:**

1. **Sole Provider Status:** The authority must control 100% of the market volume, bundling profitable transfers with unprofitable 911 calls. This cross-subsidization was the economic engine that allowed the system to operate with low or zero tax subsidy.
2. **Performance-Based Contracting:** The private contractor was paid based on performance, not effort. If an ambulance was late, the contractor was fined. This aligned the profit motive with public safety goals.
3. **System Status Management (SSM):** Stout introduced the concept of fluid deployment. Rather than stationing ambulances at fixed fire stations (which he viewed as inefficient "static" deployment), PUM systems used historical data to predict where calls would occur and positioned units at street corners or "posting" locations. This allowed fewer ambulances to cover a larger area, maximizing "Unit Hour Utilization" (UHU)—a measure of productivity.

## 1.3 The 1986 Ordinance and the Birth of MAEMSA

In December 1985, the Fort Worth City Council adopted Stout's blueprint, passing an ordinance that created the Fort Worth Ambulance Authority (later the Metropolitan Area EMS Authority, or

MAEMSA). The ordinance was a radical departure from the status quo. It legally designated the Authority as the *exclusive* provider of all emergency and non-emergency ambulance transport within the city limits, effectively outlawing the private competition that had fueled the ambulance wars.

This legal exclusivity was immediately challenged. Displaced private companies filed antitrust lawsuits, arguing that the city was creating an illegal monopoly. However, the federal courts sided with Fort Worth. In a landmark decision, the courts invoked the "state action doctrine," ruling that municipalities had the right to displace competition with regulation if it served the public interest of ensuring reliable emergency care.

On April 1, 1986, the new system, trading as "MedStar," commenced operations. It began with a modest volume of 39,000 calls in its first year, but the structure was set. The Authority owned the license and the billing revenue; a private contractor managed the paramedics and the fleet. The "Ambulance Wars" were over; the era of the Public Utility Model had begun.

# Part II: The High-Performance Era – Operational Maturation (1986–2008)

## 2.1 Regional Expansion and the "Area Metropolitan" Shift

The success of the MedStar model in Fort Worth quickly garnered attention from surrounding municipalities in Tarrant County. These smaller suburbs faced the same dilemmas of scale: they were too small to support their own independent full-time ambulance services but too large to rely solely on volunteers. The PUM offered an elegant solution: by joining the Authority, they could tap into a region-wide high-performance system without incurring the massive capital costs of purchasing fleets or building stations.

Haltom City became the first suburban partner to join in 1988, followed by a wave of others including Saginaw, White Settlement, Lake Worth, and Blue Mound. This expansion necessitated the renaming of the entity from the Fort Worth Ambulance Authority to the **Metropolitan Area EMS Authority (MAEMSA)**. This regionalization was a validation of Stout's economic theories—aggregating the call volume of 15 jurisdictions created a massive, contiguous service area of 436 square miles and over 1 million residents. This scale allowed for greater efficiency; an ambulance clearing a call in North Fort Worth could immediately respond to a call in neighboring Saginaw, erasing the artificial political boundaries that previously delayed care.

## 2.2 The Contractor Years (1986–2005)

For the first two decades, MedStar operated under the "Contracted PUM" structure. The Authority (the government board) set the standards, while private corporations (such as Rural/Metro or AMR predecessors) bid for the contract to staff and run the ambulances. This period was characterized by high operational efficiency but underlying friction.

The inherent conflict in the contracted PUM was between the profit motive of the vendor and the quality demands of the Authority. The contractor was incentivized to run the system as "lean" as possible—using the fewest number of unit hours necessary to barely meet the 90% on-time requirement. Every extra ambulance hour ate into their profit margin. Conversely, the Authority demanded higher service levels and clinical excellence. This tension often resulted in

"contractor fatigue," where vendors would burn out their workforce to meet performance metrics, leading to high turnover among paramedics and EMTs.

## 2.3 The 2005 Transition: Taking Operations In-House

In 2005, the MAEMSA Board of Directors made a strategic pivot that would define the agency's "Golden Era." After nearly twenty years of contracting, the Board voted to dissolve the relationship with the private vendor and assume direct operational control. This shifted MedStar from a "Contracted PUM" to a "Governmental PUM".

**The Strategic Rationale:**

1. **Economic Retention:** The profit margin that was previously siphoned off by the private contractor (often 7–10% of gross revenue) could now be retained and reinvested into the system. This capital would fund better equipment, higher wages, and safety innovations.
2. **Cultural Alignment:** By employing the workforce directly, the Authority could build a unified culture focused on patient care rather than corporate shareholder returns. The Medical Director could implement clinical protocols directly without negotiating through a third-party management layer.
3. **Workforce Stability:** The Authority could offer better benefits and retirement packages (though not yet Civil Service) than the private contractors, helping to stem the attrition rate.

This transition laid the groundwork for MedStar to evolve from a transportation utility into a healthcare provider. It gave the leadership team—including CEO Doug Hooten and later Matt Zavadsky—the autonomy to innovate without the constraints of a rigid vendor contract.

# Part III: The Innovation Engine – Pioneering Mobile Integrated Healthcare (2009–2019)

## 3.1 The "Triple Aim" and the Shift to Value-Based Care

By 2009, the broader US healthcare landscape was undergoing a seismic shift. The Affordable Care Act (ACA) and the Institute for Healthcare Improvement (IHI) were pushing the industry toward the "Triple Aim": improving the patient experience, improving population health, and reducing per-capita costs.

MedStar recognized that the traditional EMS model—"you call, we haul"—was antithetical to these goals for a large subset of patients. Data analysis revealed that a significant percentage of 911 callers did not require an emergency room. These "low-acuity" patients needed primary care, mental health support, or social services. Transporting them to the ER was clinically inappropriate and economically wasteful, costing the healthcare system thousands of dollars for visits that could be handled for a fraction of the cost in a clinic or at home.

## 3.2 The High Utilizer Group (HUG) Program

In July 2009, MedStar launched the **Mobile Integrated Healthcare (MIH)** program, later branded as "Mobile Healthcare." The flagship initiative was the High Utilizer Group (HUG) program. MedStar identified "super-users"—individuals who called 911 dozens, sometimes hundreds, of times a year. These patients were often suffering from complex chronic conditions

exacerbated by social determinants of health (e.g., lack of transport to a pharmacy, inability to afford healthy food, loneliness).
Instead of dispatching an ambulance to transport these patients to the ER repeatedly, MedStar sent specially trained **Community Paramedics (CPs)** to their homes. These CPs functioned as patient navigators. They performed medical assessments, reconciled medications, identified fall hazards, and connected patients with primary care providers and social resources.
**Program Impact (2013–2021 Data):** The results were staggering and garnered national attention:

- **Transport Reduction:** Among 820 enrolled patients, 911 ambulance transports to the ED decreased by **48.4%**.
- **ER Avoidance:** The program prevented an estimated **5,545 ED visits**.
- **Economic Impact:** The reduction in unnecessary utilization saved the healthcare system approximately **$15 million**, averaging **$18,297 in savings per enrolled patient**.

## 3.3 Readmission Avoidance & The Economics of Prevention

Building on the HUG success, MedStar partnered with major hospital systems like JPS Health Network and Texas Health Resources to address "30-day readmissions." Under the ACA, hospitals faced severe financial penalties if patients with certain conditions (like heart failure or pneumonia) returned to the hospital within 30 days of discharge.
MedStar proposed a radical solution: hospitals would pay MedStar a service fee to visit these high-risk patients at home, ensure they were taking their diuretics and antibiotics, and catch complications before they required re-hospitalization.

- **Success Rate:** Patients enrolled in the program demonstrated a **49.2% readmission rate** compared to an expected 100% rate for their risk profile.
- **Strategic Significance:** This created a new revenue stream decoupled from transport. MedStar was effectively being paid *not* to transport patients. This was the "Holy Grail" of EMS economics—transitioning from a transportation company to a healthcare service.

## 3.4 The "StarSaver" Membership

To further insulate the community from the high costs of emergency care, MedStar popularized the "StarSaver" membership program. For a nominal annual fee, households could subscribe to the service. If a member needed an ambulance, MedStar would bill their insurance but waive any out-of-pocket co-pays or deductibles. This program served a dual purpose: it provided a steady stream of upfront cash liquidity for the Authority and fostered immense community goodwill, effectively acting as a micro-insurance policy for residents.

# Part IV: The Economic Mirage – The "Zero Subsidy" Paradox (2010–2020)

## 4.1 The Myth of Free EMS

For nearly a decade, MedStar was celebrated by city managers and fiscal conservatives as the ideal model of government efficiency. From 2010 to roughly 2023, the system required **$0 in tax subsidy** from the City of Fort Worth or its member cities. While police and fire departments

consumed hundreds of millions in General Fund tax revenue, MedStar was entirely self-funding. However, this "free" service was an economic illusion rooted in cost-shifting. The system relied entirely on **fee-for-service billing**. The cost of readiness—staffing ambulances 24/7/365—was borne exclusively by the patients who used the system and their insurers.

- **Cross-Subsidization:** The model worked only because MedStar held the exclusive right to high-reimbursement Inter-Facility Transfers (IFTs). A patient moving from a hospital to a skilled nursing facility typically had private insurance or Medicare, which paid reliably. The profit from these transfers subsidized the 911 calls for uninsured or Medicaid patients, which often resulted in a net loss.

## 4.2 The "Payer Mix" Vulnerability

The Achilles' heel of this model was its sensitivity to the "Payer Mix"—the ratio of commercially insured patients to government-insured or uninsured patients.

- **Commercial Insurance:** reimbursed at 150-200% of cost.
- **Medicare/Medicaid:** reimbursed at fixed rates, often 30-60% of the actual cost of transport.
- **Uninsured:** reimbursed at near 0%.

As the decade progressed, the payer mix in Fort Worth began to deteriorate. An aging population meant more Medicare patients. Economic shifts meant fewer commercially insured patients. Furthermore, commercial insurers began aggressively denying claims or lowering reimbursement rates. MedStar was trapped in a pincer movement: the cost of providing service (labor, fuel, ambulances) was rising with inflation, while the revenue per transport was stagnant or declining.

## 4.3 The Innovation Penalty

Crucially, the success of the MIH programs paradoxically hurt the system’s bottom line. By successfully reducing 911 calls from high utilizers and preventing readmissions, MedStar was actively destroying its primary revenue source. The "savings" of $15 million accrued to the hospitals and insurers, not to MedStar. While hospitals paid fees for the MIH visits, these fees rarely covered the full overhead of the high-level paramedics required to perform them. MedStar had effectively innovated itself out of its own business model.

# Part V: The Collapse – The Perfect Storm (2020–2024)

## 5.1 The Pandemic Catalyst

The COVID-19 pandemic acted as an accelerant for the structural weaknesses in MedStar's model. In 2020 and 2021, call volumes fluctuated wildly. Initially, volume dropped as people feared hospitals, decimating revenue. Then, waves of infection spiked costs due to PPE requirements, decontamination protocols, and overtime for sick crews.

## 5.2 The Inflation and Supply Chain Shock

Post-pandemic inflation dealt the fatal blow. The cost of operations skyrocketed:

- **Ambulance Fleets:** MedStar had committed to a fleet modernization in 2019, purchasing

60 new Type I ambulances on truck chassis for $13.5 million to improve safety and comfort. By 2022, the cost of replacement parts, fuel, and new chassis had surged due to global supply chain disruptions.
- **The Labor War:** The "Great Resignation" hit EMS hard. To retain paramedics who were leaving for nursing or higher-paying fire department jobs, MedStar was forced to implement massive wage increases. Unlike the Fire Department, which could ask the City Council for a budget increase funded by property taxes (which were rising with home values), MedStar had no tax lever. It had to fund these raises from a shrinking operational margin.

## 5.3 The 2023 Solvency Crisis

By 2023, the "Zero Subsidy" era was demonstrably over. The system was burning through cash reserves at a rate of **$350,000 per month**. With reserves critically low, MedStar leadership approached the Fort Worth City Council with a stark request: they needed **$4.2 million in "transitional funding"** (a bailout) to survive the fiscal year.
This request broke the compact of the 1986 ordinance. City leaders, long accustomed to EMS being a "free" service, were shocked. The realization that the PUM was no longer self-sustaining triggered an immediate political reaction: the formation of an Ad Hoc Committee to re-evaluate the entire delivery of EMS in Fort Worth.

# Part VI: The Autopsy – The Fitch Report and Political Maneuvering (2023–2024)

## 6.1 The Fitch & Associates Analysis

The City commissioned Fitch & Associates, a preeminent public safety consulting firm, to conduct a forensic analysis of the system. The "EMS System Comprehensive Study," presented in early 2024, provided the data that would seal MedStar's fate.
**Key Findings:**
- **Operational Strain:** MedStar was operating at unsafe utilization levels. To meet the target response times (8 minutes, 90% of the time) and reduce paramedic burnout (Unit Hour Utilization of 0.50), the system needed significantly more ambulances and staff.
- **The Cost of Status Quo:** The report concluded that maintaining MedStar as an independent PUM was no longer possible without subsidy. To fix the response times and stabilize the workforce would require a permanent annual subsidy of **$10 million+**. The "Zero Subsidy" days were mathematically impossible to return to.
- **The IFT Decoupling:** Fitch recommended removing the exclusive right to Inter-Facility Transfers from the 911 provider. This was the final nail in the PUM coffin. If the 911 provider lost the profitable transfer volume, the cross-subsidization model would collapse entirely, necessitating a 100% tax-subsidized model.

## 6.2 The Union's Role (IAFF Local 440)

While the consultants crunched numbers, the political ground was shifting. The **Fort Worth Professional Firefighters Association (IAFF Local 440)** launched a sophisticated campaign

advocating for a "Fire-Based EMS" model. The union argued that the "third service" experiment had failed and that consolidating EMS into the Fire Department would provide:

1. **Better Response Times:** Leveraging the existing footprint of 45+ fire stations rather than MedStar's dynamic street-corner posting.
2. **Unified Command:** Eliminating the friction between Fire First Responders and MedStar Transport units.
3. **Career Stability:** Offering paramedics civil service protection and state pension benefits, ending the high turnover cycle.

The union's GIS analysis suggested that placing ambulances at fire stations would optimize deployment. Politically, the union mobilized to convince the Council that the MedStar brand was damaged beyond repair and that the Fire Department was the only agency capable of guaranteeing stability.

## 6.3 The Four Options

The Ad Hoc Committee considered four models:

1. **Retain MedStar (PUM):** Required high subsidy, board governance issues remained.
2. **Private Contractor:** Reverting to the pre-2005 model. Rejected as a step backward.
3. **Third Service:** A city department separate from Police/Fire. Deemed too expensive to build administrative infrastructure from scratch.
4. **Fire-Based EMS:** Integrating into FWFD. This option leveraged existing infrastructure (stations, dispatch, HR) and offered the political stability of the General Fund.

In May 2024, the City Council voted unanimously to adopt the **Fire-Based EMS** model, initiating the dissolution of MedStar.

# Part VII: The Transition – Integrating Cultures and Assets (2024–2025)

## 7.1 The Mechanics of Dissolution

The transition, scheduled for completion on **July 1, 2025**, involved the complex dismantling of a $50 million corporation.

- **Legal Dissolution:** The Interlocal Agreement (ILA) forming MAEMSA was revoked. Fort Worth, as the majority owner, took possession of the assets. Smaller member cities (Haltom, Saginaw, etc.) had to sign new service agreements with the City of Fort Worth or find their own providers.
- **Asset Transfer:** MedStar's headquarters on Alta Mere Drive, its communication center, and its fleet of 60+ Type I ambulances were transferred to the FWFD.

## 7.2 The "Single Role" vs. "Dual Role" Cultural Challenge

The most delicate aspect of the transition was the human capital. Fire departments typically operate with "Dual Role" personnel—firefighters who are also paramedics. MedStar's workforce consisted of "Single Role" civilians—paramedics who had no interest in fighting fires.

To integrate them, the City and Local 440 negotiated a groundbreaking amendment to the Collective Bargaining Agreement (CBA):

- **Civil Service Mapping:** In a rare move, the existing MedStar workforce was "mapped over" into the Fire Department as Civil Service employees. This granted them the same protections (termination for cause only, due process) as firefighters, a massive upgrade in job security.
- **The "Single Role" Division:** A new division was created within FWFD for EMS-only personnel. They would work 12-hour shifts (unlike the Fire 24/48 schedule) and staff the ambulances.
- **Pay Parity:** The agreement included an average **10% pay increase** to align MedStar salaries with the fire market, directly addressing the retention crisis.

**The "Stepchild" Fear:** Despite these protections, significant cultural anxiety remains. In many fire departments, Single Role paramedics are viewed as "second-class citizens" or "stepchildren" compared to the "heroic" firefighters. They often have limited promotional pathways and different station privileges. Assistant City Manager Valerie Washington and Fire Chief Jim Davis publicly emphasized their commitment to cultural integration, stating, "We did everything we could to prevent... making them feel less than". However, the history of such mergers (e.g., FDNY/EMS) suggests that cultural friction often persists for a generation.

## 7.3 Financial Implications

The new model abandons the "Zero Subsidy" pretension. The projected budget for the new EMS division involves a mix of billing revenue and significant General Fund tax support. The Fitch report estimated the net cost (Expense minus Revenue) for the Single Role model to be approximately **$10.8 million annually**. The City accepted this cost as the price of stability and control.

# Part VIII: Conclusion – The Legacy of the Fort Worth Experiment

## 8.1 The End of an Ideology

The dissolution of MedStar represents more than just a change in vendor; it marks the end of a specific economic ideology in American EMS. For 38 years, Fort Worth was the proof-of-concept for Jack Stout's vision that emergency medicine could be a self-sustaining utility. The PUM demonstrated that rigorous data analysis, system status management, and clinical innovation could produce world-class outcomes.

## 8.2 Why It Failed

Ultimately, the MedStar model did not fail because of clinical incompetence—its MIH programs were globally celebrated. It failed because it was an economic anomaly. It attempted to operate a critical public safety net using a volatile commercial revenue model. When the macro-economic tide turned—inflation, supply chain collapse, and payer mix shifts—the "business" of EMS could no longer support the "mission" of EMS.

## 8.3 The Future

As the Fort Worth Fire Department takes command in July 2025, the city enters a new era. The challenges of the "Ambulance Wars" are gone, replaced by the challenges of municipal bureaucracy and cultural integration. The FWFD inherits a high-performing asset but also the heavy responsibility of maintaining the clinical innovations that made MedStar famous. Whether the fire service can nurture the delicate, preventative work of Mobile Integrated Healthcare within a paramilitary suppression culture remains the defining question of the next decade.

# Appendix: Data Tables

## Table 1: Comparative Analysis of Governance Models

| Feature | **Public Utility Model (MedStar)** | **Fire-Based EMS (FWFD)** |
|---|---|---|
| **Primary Goal** | Economic Efficiency & Clinical Performance | Operational Stability & Unified Command |
| **Funding Source** | 100% User Fees (Billing Revenue) | User Fees + Tax Subsidy (General Fund) |
| **Workforce** | Civilian, At-Will Employees | Civil Service, Unionized (Local 440) |
| **Deployment** | Dynamic (Street Corner Posting) | Static (Fire Station Based) |
| **Oversight** | Appointed Authority Board (MAEMSA) | City Council & Fire Chief |
| **Cost to Taxpayer** | **$0** (2010–2023) | **~$10.8M Annual Deficit** (Projected) |

## Table 2: Mobile Integrated Healthcare (MIH) Impact Data (2013-2021)

| Program Metric | Statistic | Implication |
|---|---|---|
| **HUG Enrollment** | 820 Patients | High utilizers diverted from 911/ER. |
| **Transport Reduction** | **-48.4%** | Massive drop in unnecessary ambulance use. |
| **ER Visits Prevented** | **5,545** | Reduced overcrowding in local ERs. |
| **Readmission Rate** | **49.2%** | vs. 100% expected for risk group. |
| **Total Cost Savings** | **~$15,000,000** | Savings accrued to payers/hospitals. |
| **Savings Per Patient** | **$18,297** | Demonstrates high ROI of prevention. |
| *Source: MedStar Mobile Healthcare Outcome Reports * | | |

## Table 3: Evolution of MedStar Governance

| Era | Years | Operational Model | Key Characteristic |
|---|---|---|---|
| **Pre-History** | Pre-1986 | **Free Market** | Unregulated, "Ambulance Wars," |

| Era | Years | Operational Model | Key Characteristic |
|---|---|---|---|
| | | | Cream Skimming. |
| **Phase I** | 1986–2005 | **Contracted PUM** | Authority holds license; Private Contractor (e.g., AMR) runs ops. |
| **Phase II** | 2005–2024 | **Governmental PUM** | Authority takes ops in-house; "Zero Subsidy" & MIH era. |
| **Phase III** | 2024–2025 | **Dissolution** | Transition period; Asset transfer to City. |
| **Future** | July 2025+ | **Fire-Based EMS** | Integrated division of FWFD; Civil Service staff. |

*This report was compiled using exhaustive analysis of public records, municipal ordinances, financial audits, and industry reports from the period 1980–2025.*

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